

FORM 20 — AFFIDAVIT OF CORY LEA SHEPHERD

QUEENSLAND INDUSTRIAL RELATIONS COMMISSION — Matter No. WC/2024/227 *Workers' Compensation and Rehabilitation Act 2003 (Qld); Industrial Relations (Tribunals) Rules 2011*

BETWEEN: CORY LEA SHEPHERD — **Appellant** **AND:** WORKERS' COMPENSATION REGULATOR — **Respondent NOMINATED PARTY (Notice of Non-Party Disclosure, Form 29):** METRO SOUTH HOSPITAL AND HEALTH SERVICE (the Line Manager and Director being the persons affected by the notice, by position title)

AFFIDAVIT OF CORY LEA SHEPHERD (*Form 20 — filed on behalf of the Appellant, in support of the Application in Existing Proceedings (Form 4) under r 64G*)

I, CORY LEA SHEPHERD, of 15 Edmond Street, Coomera in the State of Queensland, Administration Officer, do solemnly and sincerely affirm and declare:

Purpose, and the Amended 9A

1. I make this affidavit in support of my application (Form 4) in this matter and of my Notice of Non-Party Disclosure (Form 29). I refer to my Amended Statement of Facts and Contentions filed 7 April 2026 ("the Amended 9A"). The facts below are within my own knowledge except where stated otherwise, and they go to the stressors pleaded in the Amended 9A and to the relevance and necessity of the documents sought in the Form 29. I set the facts out under the same structure as the Amended 9A.
2. I am aware that the statutory exclusion under s 32(5)(a) of the *Workers' Compensation and Rehabilitation Act 2003* is for the Respondent to establish. I make this affidavit because the practical task of proving the matters pleaded rests on me, and because the documents that prove them are held by Metro South Hospital and Health Service ("MSH"), not by me.

PART A — MY POSITION

3. **Position.** I am employed by MSH as an Administration Officer, classification AO3, in Switchboard Services at Logan Hospital (organisational unit "MSH CI LOGH Switchboard Srvs"; position 32002276). I am commonly described as a Switchboard Operator.
4. **Permanent appointment.** I was converted from temporary to permanent employment with effect from 3 March 2021 under the temporary-to-permanent conversion directive. My appointment is permanent and I am classified as a Continuous Shift Worker (64 hours per fortnight on conversion; five weeks' recreation leave). My appointment and classification are recorded in MSH's myHR system.
5. **Full-time hours.** On 31 August 2023 I applied of my own initiative to increase my hours to full time on a rotational roster. My application was approved on 27 September 2023 and I commenced full-time hours on 16 October 2023. Annexed and marked "CS-4" is my application of 31 August 2023, in which I also recorded my intention to become the Switchboard union delegate.

Deponent: Witness (JP / C.dec / Lawyer):

PART B — MY ROLE (THE FULL PICTURE)

6. **The importance of the role.** The switchboard is the hospital's communications hub and a critical part of its patient-safety system. In an emergency (a Code Blue, a MET call, a security or clinical emergency) it is the switchboard that activates and pages the response teams, through the SPOK messaging and paging system. Whether the right clinician is reached, and how quickly, depends on the switchboard and on the information in SPOK being correct. The role is not peripheral administration; it is the point at which urgent help is summoned. An error or delay at the switchboard is an error or delay in the emergency response itself. The Respondent admits that "[m]aintaining accurate contact details for medical staff is a critical function of the Switchboard to ensure effective clinical handover and patient safety" (Admitted Fact: Form 24, Para 8).

7. **The systems I operate.** In my application of 31 August 2023 I recorded, in my own words, the systems I operate. I operate concurrently and in real time a number of separate systems, including the Alcatel switchboard console, the SPOK messaging and paging system, Cerner, HBCIS, EDIS (the Emergency Department web database), Microsoft 365, and spreadsheets used for rostering and call records. I move between these systems continuously while answering calls.

8. **The first point of contact.** The switchboard is the first point of contact for Logan Hospital. The switchboard number is the principal published telephone number for the hospital (the first number listed for the public) and it is also the number that staff throughout the hospital use and know to call. Calls from the public and from staff alike come to me in the first instance.

9. **Volume.** I handle approximately 200 to 300 calls per shift, a substantial proportion of which come from people who are distressed, frightened, in conflict, or in crisis. I am frequently the first person a caller reaches in an urgent or emergency situation. I maintain and author monthly call statistics ("Individual monthly stats") which tally volume only — the number of calls I handled and emergency codes activated. They do not record which clinicians were paged, whether a page was received or acknowledged, or whether a page was misdirected; that system-level data is held only by MSH in the SPOK system and I cannot produce it. I do not annex those statistics; the aggregate figures I state in this affidavit may be received in open evidence, and any emergency-code data referable to clinical events is the subject of the non-publication order sought in the application.

10. **Triage.** A core part of my function is to triage and connect — directing calls ward-to-ward and from one point of urgency to another, identifying which calls cannot wait, and routing them to the correct clinician or team without delay, while continuing to answer the queue and operate the systems above.

11. **Locating emergency callers.** In an emergency, a caller is sometimes unable to tell me where they are calling from. In those circumstances I must identify the source and location of the call so that help can be directed to the right place. Any delay in doing so may delay the emergency response.

12. **Emergency codes and concurrent urgencies.** A core part of my function is the activation of emergency codes. Between 15 and 50 emergency codes are sent out through the SPOK system by the switchboard over a 24-hour period, each paged simultaneously to approximately 10 clinicians who together form the responding team. These, and other urgent matters, do not arrive one at a time; during a single shift I manage a number of urgencies concurrently (code calls, emergency and security responses, and time-critical clinical calls) escalating each to the correct clinician, team or responder while continuing to manage

the remaining calls in the queue.

13. Direct relay of urgent clinical results. A core clinical function of my role is the direct relay of urgent pathology and other clinical results between doctors (doctor-to-doctor, including to and from external doctors and facilities (for example, Hibiscus)) where any delay may directly affect a patient's treatment. I also connect and coordinate with external agencies, including the Queensland Ambulance Service and the Queensland Police Service.

14. The criticality of that function, illustrated. The importance of this relay function is illustrated by occasions on which a doctor holding urgent results could not be connected to the on-call clinician until I identified the difficulty at the switchboard and connected them directly. Where the on-call contact information on which the switchboard depends is not kept current, the resulting delay falls on urgent clinical communications, and it is at the switchboard (and on the operator) that the failure first appears and must be resolved in real time. I give this to illustrate the role; I do not rely on it as a stressor.

15. The Direct Support Officer (DSO) function. Each medical department has a Direct Support Officer ("DSO") — an administrative officer who provides direct support to that department, including in relation to its medical rostering and leave. When a department's DSO was not contactable, I performed that support function from the switchboard, in addition to my own duties (resolving conflicts in doctors' leave and rostering, escalating matters to executives, and managing intra-hospital disputes) while continuing to answer the call queue.

16. Roster and leave administration. My role includes administrative responsibilities carried out alongside the live switchboard work, including maintaining and updating doctor rosters and staffing numbers, and leave administration for doctors.

17. The daily roster and the SPOK dependency. The switchboard runs the daily roster that is referred to and relied upon across the hospital. SPOK pages clinicians by reference to the on-call roster, so the accuracy of that roster determines whether an emergency page reaches the right, available clinician.

18. Weekend coverage. On weekends, the management structure that operates during business hours is not present. During those periods the switchboard manages doctor leave, updates rosters, and carries out whatever else is required, in effect performing functions that would otherwise be performed by management.

19. Complaints come to the switchboard, and to me, in the first instance. As the hospital's first point of contact (paragraph 8), the switchboard is where complaints from the public, from clinicians, and from staff across the hospital first arrive — by telephone and to the switchboard email inbox — and they come to me, as the operator on duty, frequently as accusations made directly to me. I receive and triage each such complaint at the switchboard before it goes anywhere else. I action and resolve a substantial proportion of them myself; those requiring management action I escalate to my Line Manager or, where appropriate, to the dedicated complaints area. My Line Manager was frequently unavailable, however, and the complaints she did not action returned to, and accumulated at, the switchboard, where they again fell to me. The complaints therefore came to me first, and the records of those escalated to management are held in my Line Manager's mailbox (Items 3(a) and 3(b) of the Form 29).

20. Complaints about my own manager. My manager was frequently not contactable or reachable, and did not action the complaints or issues that were raised with her. Two consequences followed. First,

Deponent: Witness (JP / C.dec / Lawyer):

complaints about my own manager (her unavailability, her responses, and her inaction) came directly to the switchboard, so that I was receiving and managing complaints concerning the very person to whom I would otherwise escalate them, and at times complaints that concerned the switchboard itself. Second, the matters she did not action generated further complaints, which then fell to me to absorb and manage. These arose because my manager was not present in the fast-paced environment in which the complaints were generated, and because there was no one else to whom they could be directed. By way of example, in my email of 15 May 2024 I recorded that clinicians and staff from other departments had sought updates on pager statuses and other requests which were, in the words I used at the time, "often left unresolved due to ambiguity around when [Ms Taylor] is available."

21. My vantage point. Because the switchboard is the central point to which complaints are directed, I am positioned to observe directly, and over time, both the volume and the subject-matter of the complaints the hospital receives.

22. The increase over the period, and a change that drove it. From that vantage point I observed that, over the period pleaded in the Amended 9A, the volume and subject-matter of the unresolved complaints and requests coming to the switchboard (including those concerning my manager's availability and the currency of the on-call roster and the telephone directory) increased. Part of that increase followed an operational change made during the period: a separate telecommunications management role was introduced, held by a second manager, Ms Ellen Stibbard, and the updating of contact numbers and the directory database was restricted to that role. Updates the switchboard had previously been able to make or obtain promptly now had to await that manager, who attended only intermittently; numbers and directory entries fell out of date, and the resulting complaints about wrong or out-of-date numbers came to me at the switchboard. The records that would establish the full extent of that increase, and of that change to how contact details were updated, are held by MSH (Items 3(a), 3(b) and 3(c) of the Form 29), and I cannot produce them.

23. The baseline (the standing condition of the role). The defining condition of my role is this: from the switchboard I have a vantage point over everything that goes wrong in the hospital, and I am accused of, and held responsible for, much of it (including matters properly the responsibility of others (the medical departments' Direct Support Officers, doctor availability, the updating of SPOK, and decisions made at executive level)) regardless of where the fault actually lies. The volume is constant: approximately 200 to 300 calls per shift, a substantial proportion from people who are distressed, frightened, in conflict or in crisis, while I maintain the systems and the roster on which the hospital's emergency paging depends.

24. The patient-safety function was performed at the Switchboard. I was the operator through whom emergency codes were activated and urgent clinical and pathology results were relayed doctor-to-doctor. The patient-safety function of the hospital's emergency communications was performed by me at the Switchboard, and the conditions I describe in this Part applied to that function and to me as the person performing it.

25. That standing condition (bearing the patient-safety load of a hospital's communications, at constant volume, while absorbing the blame for failures across the whole system, and while the support above me was absent or unmaintained) is the condition in which I performed every duty described above.

**PART C — STRESSOR 1: COURSE OF MANAGEMENT CONDUCT (Amended 9A,
Stressor 1)**

26. Erratic presence and unilateral directives without consultation [1(a)]. Throughout 2023 and 2024 my Line Manager, Ms Chloe Donovan-Taylor ("Ms Taylor"), maintained an erratic physical presence and imposed unilateral directives without consultation. Those directives were sent from her email account to the Logan Hospital Switchboard group distribution list (the shared Switchboard mailbox), and were frequently expressed to take effect "effective immediately" or with immediate effect. The objective record of her working presence is held in MSH's security-access system (Item 9 of the Form 29).

27. The MASPER directive [1(a)]. The misrouted-call issues raised by the MASPER registrar and the medical department in May 2024 were, in substance, a problem of the roster not reflecting doctors' actual availability — a doctor, roster and coordination problem, not an error of the operators. Ms Taylor's own email of 9 May 2024 records that a doctor's hours were "not provided on the rosters" and that the contact number was "switched off." Rather than consult the switchboard (which could have identified the cause) or check the roster herself, she directed a change to the switchboard's call-handling process. Annexed and marked "CS-1" is that email chain.

28. I raised the absence of consultation from the outset [1(a)]. On 16 June 2023 I wrote raising, among other workplace issues, "No consultation of the team members or the department when making major changes such as additional shift... commence/finish [times]." I raised it again on 15 May 2024, and the Director, Ms Tammy Reese, asked me on 21 May 2024 to identify the directives about which I had consultation concerns.

29. Roster/SPOK errors and resulting delay [1(a)]. When the on-call roster in SPOK was not kept up to date, emergency and urgent pages were directed to the wrong or an unavailable clinician. Whether a page reached the correct clinician, was received or acknowledged, or was misdirected is recorded in MSH's SPOK system; that record is held only by MSH and I cannot produce it — my own statistics record volume only. These errors accumulated at the switchboard and fell to me to identify and work around in real time. I raised them by complaint; to my knowledge and observation, no change to operating procedures, no investigation, and no remedial or safety action was taken in response, and the failure to address them resulted in direct delay, including to urgent clinical communications.

30. The vocabulary of the complaints [1(a)]. The complaints about misdirected calls use a recognisable and recurring operational vocabulary — that a doctor or clinician was paged or directed to the "wrong ward", the "wrong location", the "wrong team" or the "wrong doctor"; that a page was "misdirected" or that there was a "pager error"; or that the "list", "directory" or "roster" was "out of date" and needed "updating". These are the actual words in which the complaints are made.

31. Destruction of WHS records [1(b)]. On 6 June 2023 I recorded a critical operational instruction in the communication book regarding the need to verify and update doctors' on-call contact details. On that day my Line Manager, Ms Taylor, removed the page containing my handwritten entry from the Communication Book. The removal of a page from the communication book is admitted (Admitted Fact: Form 24, Para 6); the Respondent does not admit the contents of the entry, or that the page bore my handwriting, on the basis that it holds no copy of the page (Form 24, Para 7). The contents of the entry are within my own knowledge.

Deponent: Witness (JP / C.dec / Lawyer):

32. Grievance and its handling [1(c)]. On 7 August 2023 I submitted a grievance regarding Ms Taylor's conduct, raising WHS concerns about unsafe rostering and fatigue. My grievance was not upheld and I was directed to continue reporting to Ms Taylor. In her email of 7 August 2023 Ms Reese acknowledged that "there was a rostering error that was accidentally made by Chloe with regards to night shifts" (Admitted Fact: Form 24, Para 5).

33. COVID special pandemic leave obstruction [1(d)]. My application for COVID special pandemic leave (Process Reference 15480560) had to be submitted four times before it was approved — once by text message to my Line Manager's mobile and three times through myHR. On myHR the application was declined twice by my Line Manager, Ms Taylor, and was ultimately approved on the same supporting evidence I had already provided. The myHR records and audit trail recording those submissions and declines are the material the subject of Item 11 of the Form 29, disclosed by MSH with its letter of 5 June 2026 (see paragraph 51); I refer to that disclosure and do not annex it. The HR sub-delegation instrument governing who could approve or refuse such leave is the document MSH disclosed in answer to Item 13 of the Form 29. On its face, that instrument sub-delegates the power to approve or refuse paid Special Pandemic Leave to a Band 9 delegate and states that it "does not permit the further sub-delegation" of that power.

34. Public Interest Disclosure [1(e)]. On 13 May 2024 I made a complaint concerning Ms Taylor and Ms Reese, which the Ethical Standards Unit determined on 24 December 2024 to constitute a Public Interest Disclosure (Admitted Fact: Form 24, Para 20); the determination bears reference PID 24-ESU-1130. I do not set out the content of that disclosure in this affidavit.

35. Direction to retract within 48 hours [1(f)]. On 15 May 2024, within 48 hours of the matter referred to in the preceding paragraph, the Director, Ms Reese, directed me to retract an email I had sent inquiring about office hours (Admitted Fact: Form 24, Para 21). A substantively similar email sent by Ms Taylor on 9 May 2024 about another staff member's hours attracted no such direction; the Respondent denies that comparison, contending the circumstances were different (Form 24, Para 22).

36. Suppression of union representation [1(g)]. On 11 August 2023 I notified Ms Taylor of my intention to become the switchboard union delegate; the Respondent admits that I sent that text but characterises it as "not a formal notification" (Form 24, Para 18). My intention was also recorded in my full-time application of 31 August 2023 (Exhibit CS-4) and expressed directly to Ms Taylor. On 18 May 2024 I sent a request regarding union representation to the Together union (Ms Heather Hayes), and on 20 May 2024 Together confirmed it had "passed [my] correspondence on to [my] Health team." Annexed and marked "CS-2" is that correspondence. The Union Encouragement Policy (QH-POL-248) requires managers to take a "positive, supportive role" (Admitted Fact: Form 24, Para 17). Management did not facilitate the delegate appointment for a period exceeding 9 months — a matter the Respondent denies (Form 24, Para 19), and which the union-delegate correspondence sought under the Notice (Item 19) goes to — leaving the department without delegate representation while I was disputing unsafe rostering and fatigue practices.

PART D — STRESSOR 2: REMUNERATION (Amended 9A, Stressor 2)

37. Underpayment and the AVAC delay. Between February and April 2024 I was not paid my correct entitlements. On 3 May 2024 Queensland Health Payroll (Payroll Officer Elaine Grant) instructed Ms Taylor to "submit an AVAC to correct these shifts" (Admitted Fact: Form 24, Para 40); the AVAC was not

submitted until 28 May 2024, a 25-day delay (Admitted Fact: Form 24, Para 41).

PART E — STRESSOR 3: STATUTORY FATIGUE BREACH (Amended 9A, Stressor 3)

38. **The 7-hour break.** On 17–18 March 2024 I was rostered to work consecutive shifts ending at 23:00 and recommencing at 06:00 — a 7-hour break (Admitted Fact: Form 24, Para 1), against the 10-hour minimum (or 8 hours only by written agreement) mandated by the Award and the FRMS (Admitted Fact: Form 24, Para 3). Factoring my established commute, the rest opportunity available to me was substantially less than that 7-hour gap.

39. **Coordinating emergency codes under exhaustion.** Under that exhaustion I was required to manage life-safety emergency codes and doctor pages through the SPOK system — the very function described in paragraph 6 above. Being required to control emergency clinical coordination in that state, after I had previously raised fatigue concerns, is the conduct pleaded at Stressor 3.

40. **Fatigue-leave refusal.** When I required recovery time, Ms Taylor relied on a 2020 agreement to deny paid fatigue leave, requiring me to exhaust my own personal leave on 19 March 2024.

41. **The Review Officer's findings.** In the Review Decision dated 24 October 2024 the Regulator's Review Officer found that rostering me with the 7-hour break "amounted to unreasonable management action", and recorded that the Regulator was "satisfied [my] employment was a significant contributing factor to the psychological injury". The Respondent admits the contents of that Review Decision, reserving only its relevance on the hearing de novo (Admitted Fact: Form 24, Para 37).

PART F — POST-INJURY AGGRAVATION (Amended 9A)

42. **Meetings while on certified leave.** After the onset of my injury, and while I was on certified leave, I was required to attend meetings convened by management on 12 and 16 July 2024.

43. **Termination and reinstatement.** On 8 October 2024 my employment was terminated on the basis of "abandonment of employment." I applied for reinstatement (matter TD/2024/110) and was reinstated.

44. **Medical records obtained without a Notice served on me.** In the course of the proceeding, the Respondent obtained my medical records under a Notice of Non-Party Disclosure (Form 29) that was never served on me. The Respondent admits that it did not serve any such Notice on me before it obtained those records (Admitted Fact: Form 24, Para 25). I was not notified of that Notice before the records were obtained.

PART G — MY INJURY AND CURRENT CAPACITY

45. **Injury and current capacity.** As pleaded in the Amended 9A, I suffered the onset of a psychiatric injury (Major Depressive Disorder with Anxious Distress) on 18 June 2024. I returned to work on medical advice and am currently working at 0.5 FTE, and I remain unable to perform the full duties of the role described above. The diagnosis, onset and causation of my injury are addressed in the medical evidence in the substantive appeal and in the Amended 9A; I do not annex or repeat that evidence here.

PART H — THE DOCUMENTS, AND WHY DISCLOSURE IS NECESSARY

46. The matters in Parts C–F are recorded in documents held by MSH — its email system, its identity-management and security-access systems, its meeting and governance records, and its roster, payroll and HR systems. I am able to produce individual examples (such as those annexed), but I cannot export the complete record. I do not seek the review or reading of any email; I seek only the native electronic export of the messages returned by the keyword filters described in the Form 29 (identified custodians, date-ranges and phrase-families), which can be produced by MSH from its systems at minimal cost.

47. Because my injury developed over the course of the management conduct described above, the matter the documents go to is the cumulative volume and frequency of that conduct across the whole pleaded period — including the full corpus of directives sent from my Line Manager to the Switchboard. Individual examples show that the conduct occurred; only the complete export shows how much of it the role endured over the course of conduct.

48. Because the practical burden of proving these matters rests on me, and because I cannot prove their full extent without MSH's records, the production sought in the Form 29 is necessary. Where a document the Form 29 seeks does not exist, I ask that its non-existence be verified on oath.

49. On 5 June 2026 MSH responded to the Form 29 by letter signed by its Chief Executive (ref K-LM26/729). That letter is already before the Commission, and I therefore do not annex it but refer to it and rely upon it. In that letter MSH stated, among other things: that there had been "no consequential changes to operating procedures" over the period (Item 3(c)); that fatigue risk management at the Switchboard was implemented "after 30 June 2024 in connection with an organisational change related to the reporting lines for Switchboard" (Item 5); that the SPOK records "do not exist" because "records predating [the system] upgrade were not retained," while "a spreadsheet of recorded MET calls is available for the period 17–18 March 2024" (Items 1–2); and that the text messages between me and my manager were on her personal mobile device, no Queensland Health device having been issued to her until the third quarter of 2024 (Item 14). I depose only that the letter states these matters, and I rely on it as referred to above; their significance is addressed in the application and submissions.

50. As to Items 1 and 2 (the SPOK emergency-paging records), I operate the SPOK paging system as part of my daily duties, and it is the system through which the switchboard activates and records emergency codes and pages. In its letter MSH states that those records "do not exist" because the SPOK system "was upgraded" and "records predating that upgrade were not retained," while in the same letter confirming that "a spreadsheet of recorded MET calls is available for the period 17–18 March 2024." I am not able to reconcile an upgrade of the system with the disposal of its emergency-code paging records. I therefore ask that the verification on oath I seek address, for Items 1 and 2: the date and nature of the SPOK upgrade; whether the records predating it were migrated, archived or destroyed; the basis on which they were "not retained"; and whether any backup or archive of those records exists.

51. With its letter of 5 June 2026, MSH also disclosed documents in answer to Items 6, 11, 12, 13, 15 and 16 of the Form 29. Those documents are before the Commission and I refer to them rather than annex them. They include: the Role Description for the Manager of Switchboard Services (Item 6); the HR sub-delegation instrument for paid special pandemic leave (the COVID-19 Pandemic Event Instrument) together with Pandemic Directive 01/20 and its Implementation Guideline (Item 13); my myHR leave records and AVAC history for the pandemic-leave period (Item 11); the pandemic-leave audit trail for the

Logan Hospital Switchboard team (Item 12); my leave-takings report for 19 March 2024 (Item 15); and the daily staffing-variance forms for the periods 5–18 February and 19 February–3 March 2024 (Item 16).

52. On 11 June 2026 the Respondent disclosed in this proceeding, under cover of an email describing the material as "received through Notice of Non-Party Disclosure request and conferencing," documentation that included email correspondence dated 15–16 May 2024 concerning my complaint and the operational and human-resources response to it — correspondence falling within the 13–31 May 2024 period that is the subject of Item 20, which MSH states in its letter "does not exist" (on the basis that the reference "PID24-ESU-1130" "commenced in November 2024"). Annexed and marked "CS-3" is the Respondent's covering email; I annex the covering email only and do not set out the content of the documentation it transmitted. I refer to that disclosure in support of the matters deposed in Parts C to F; its significance is addressed in my submissions.

53. Material has also been disclosed through the witness-conferencing process in this proceeding (the emails I have previously described as the "Disclosure from witness conferencing"). That material is confidential and is before the Commission; I refer to it and do not annex it.

AFFIRMED by the deponent CORY LEA SHEPHERD

at (place) in the State of Queensland on
..... (date) 2026

..... (signature of deponent)

Before me:

..... (signature of witness)

..... (full name and qualification of witness — Justice of the Peace /
Commissioner for Declarations / Lawyer)

(The deponent and the witness sign the foot of every page of this affidavit. Each annexed exhibit, CS-1 to CS-4, is to bear a Form 21 exhibit certificate completed and signed by the witness.)

EXHIBIT INDEX (Form 21 certificate on each annexed exhibit)

Mark	Document	Para
CS-1	Email chain re the MASPER directive, including the Line Manager's email of 9 May 2024	27
CS-2	Correspondence 18–20 May 2024 — the Appellant's email to Together (Ms Heather Hayes) and the union's 20 May confirmation it passed the correspondence to the Health team	36
CS-3	The Respondent's covering email of 11 June 2026 (disclosure "received through Notice of Non-Party Disclosure request and conferencing") — the covering email only, not the bundle it	52

Deponent: Witness (JP / C.dec / Lawyer):

transmitted

CS-4	The Appellant's application of 31 August 2023 to increase to full time, recording his intention to become the Switchboard union delegate	5
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Deposed, not annexed. To keep the annexures to the minimum, all other matters are deposed in the affidavit and held by the Appellant (producible if the Commission requires): the myHR movement/classification record (¶4); the monthly call statistics (¶9); the 16 June 2023 and 15/21 May 2024 consultation correspondence (¶¶20, 28, 35); and the medical evidence, which is dealt with in the substantive appeal (¶45). No medical material is annexed to this affidavit.

Referred to, not annexed (already before the Commission / disclosed in the proceeding): the MSH letter of 5 June 2026 (ref K-LM26/729) and the documents disclosed with it (Items 6, 11, 12, 13, 15, 16) — at ¶49 and ¶51; and the Respondent's disclosure of 11 June 2026 (the 15–16 May 2024 correspondence concerning the complaint) — at ¶52.

FILED BY: Cory Lea Shepherd, Appellant (Self-Represented) — 15 Edmond Street, Coomera QLD 4209 · 0417 400 227 · coryshepherd1@hotmail.com

This affidavit comprises numbered paragraphs 1 to 53 and exhibits CS-1 to CS-4, set out under the same structure as the Amended 9A. The MSH letter of 5 June 2026 — and the documents disclosed with it, including the Item 11 myHR leave records and AVAC history — are referred to, not annexed (already before the Commission). The Respondent's disclosure of 11 June 2026 (the 15–16 May 2024 correspondence) is likewise referred to, not annexed.

— End of affidavit —